THE Open GROUP

TOGAF® Standard – Capability Assessment

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Barangay San Antonio Health Department Enterprise Architecture

Barangay San Antonio Health Center, Pasig City

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Tracking Information

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1. Purpose of this Document

Before embarking upon a detailed Architecture Definition, it is valuable to understand the baseline and target capability level of the enterprise. This Capability Assessment can be examined on several levels:

- What is the capability level of the enterprise as a whole?

  Where does the enterprise wish to increase or optimize capability? What are the architectural focus areas that will support the desired development of the enterprise?

- What is the capability or maturity level of the IT function within the enterprise?

  What are the likely implications of conducting the architecture project in terms of design governance, operational governance, skills, and organization structure? What is an appropriate style, level of formality, and amount of detail for the architecture project to fit with the culture and capability of the IT organization?

- What is the capability and maturity of the architecture function within the enterprise?

  What architectural assets are currently in existence? Are they maintained and accurate? What standards and reference models need to be considered? Are there likely to be opportunities to create re-usable assets during the architecture project?

- Where capability gaps exist, to what extent is the business ready to transform in order to reach the target capability?

  What are the risks to transformation, cultural barriers, and other considerations to be addressed beyond the basic capability gap?

1.1 Output and Input

The TOGAF® Content Framework (see Architecture Deliverables) identifies deliverables that are produced as outputs from executing the Architecture Development Method (ADM) cycle and potentially consumed as inputs at other points in the ADM.

This deliverable is produced, updated, and consumed by the ADM Phases:

Deliverable	Output from…	Input to…
Capability Assessment	A, E	B, C, D, E, F

2. Business Capability Assessment

2.1 Capabilities of the Business

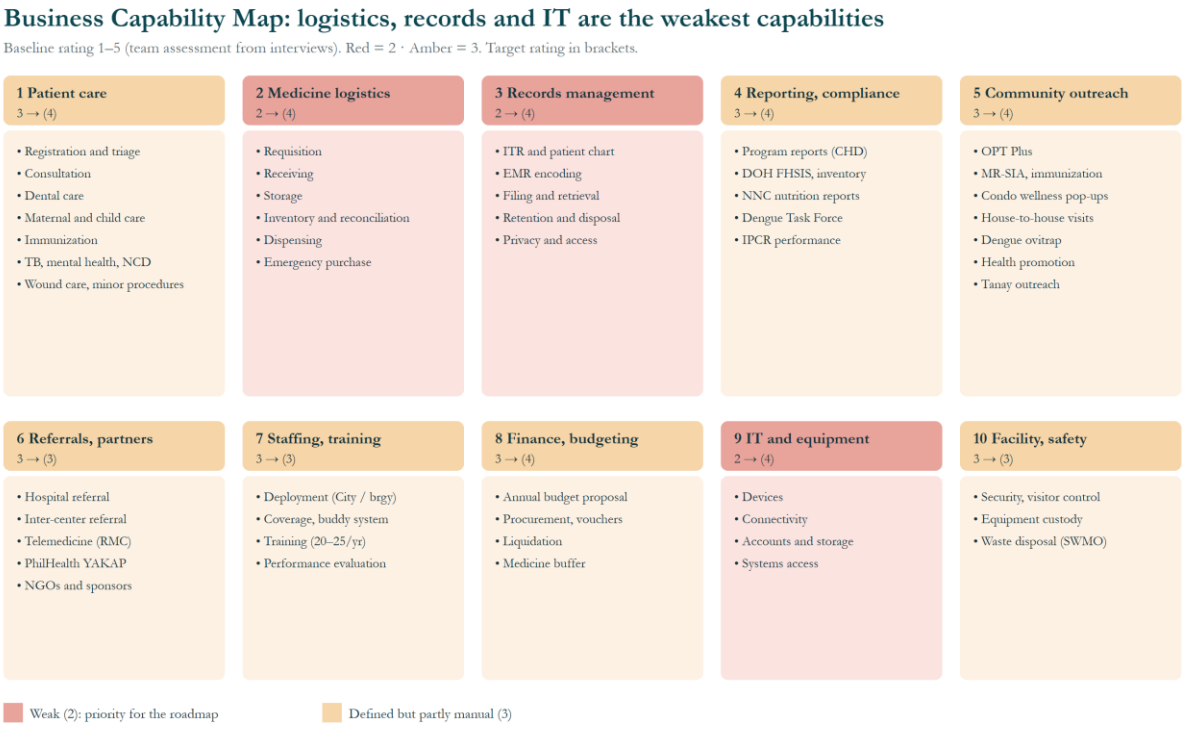


Figure 1. Business Capability Map.

How to read it: each of the ten boxes is a level-1 capability; the bullets inside are the abilities it contains. The rating shows baseline → (target) on a 1–5 scale, assessed by the team from the interviews. Red = 2 (weak), amber = 3 (defined but partly manual).

- **Weakest (2 → 4):** medicine logistics (no stock alerts, partial deliveries), records management (~8,000 mixed charts, misfiling) and IT and equipment (10-year-old laptops, no frontline computers, personal accounts).
- **Working but manual (3 → 4):** patient care, reporting and compliance, community outreach, finance and budgeting.
- **No change planned (3 → 3):** referrals, staffing and training, facility and safety; these work adequately or are controlled by the City.
- **Takeaway:** the ratings set the roadmap priority: WP1 for logistics, WP4 for records, WP2, WP3 and WP7 for IT.

#	Capability	Description
1	Patient care	Consultation, dental, prenatal, immunization, family planning, TB, mental health, wound care
2	Medicine and supply logistics	Request, receive, store, reconcile and dispense medicines and supplies
3	Records management	ITR, patient charts, EMR encoding, retention and disposal

#	Capability	Description
4	Reporting and compliance	Program reports to CHD, DOH and NNC; IPCR
5	Community outreach	OPT Plus, MR-SIA, condo wellness pop-ups, house-to-house visits, dengue ovitraps, Tanay
6	Referrals and partnerships	Hospitals, other health centers, Rizal Medical Center telemedicine, PhilHealth YAKAP
7	Staffing and training	City and barangay deployment, buddy system, 20–25 trainings a year
8	Finance and budgeting	Barangay health and nutrition budget, procurement, liquidation
9	IT and equipment	Laptops, internet, systems access
10	Facility and safety	Security, equipment custody, waste disposal

2.2 Baseline Performance Assessment

Measure	Baseline	Source
Patient visits	657 last month (360 consultations + 297 medicine pickups)	Health center verification, Oct 22
Maintenance-medicine patients	About 200–300 a month	Nurse Hardman
Referrals	About 30 a month	Nurse Hardman
Target achievement	80–90% of own caseload; pregnancies 60–70 of 181	Dr. Bonilla; Ms. Abong
Record retrieval	2–3 minutes filed; up to 10 minutes misfiled; about 5 seconds in the EMR	Ms. Abong
Medicine supply	Three months without delivery; about ₱150,000 barangay-funded in 2026	Ms. Abong; verification
Internet	About 5 outages a month	Dr. Bonilla
Late reports	None to date (late reports lower IPCR ratings)	Ms. Abong

2.3 Future Performance Aspiration

- No maintenance-medicine stock-outs.
- Each service entered once at the point of care.
- Old-patient records found in under a minute.
- Monthly reports compiled within working hours and submitted digitally.
- Higher coverage of condo residents and Tanay families.

2.4 Baseline Capability Assessment

Rating scale (team assessment from interviews): 1 Ad hoc · 2 Basic, mostly manual · 3 Defined but partly manual · 4 Managed and digital · 5 Optimized.

Capability	Baseline	Basis
Patient care	3	Defined clinical flow; no queuing; paper and EMR in parallel
Medicine and supply logistics	2	Manual utilization logs; partial deliveries; no alerts
Records management	2	~8,000 charts mixed active/inactive; misfiling; EMR depends on internet
Reporting and compliance	3	Always on time, but by manual compilation and hard copies
Community outreach	3	Well organized; condo access limited

Capability	Baseline	Basis
Referrals and partnerships	3	Clear routes; paper-based
Staffing and training	3	Frequent training; thin clinical staffing
Finance and budgeting	3	Approved budgets; manual ledger; no medicine buffer policy
IT and equipment	2	Old laptops; no computers for aides and BHWs; personal accounts
Facility and safety	3	Custodian and security in place; LGU waste pickup twice a month; records room partly secured

2.5 Future Capability Assessment

Capability	Target	What changes
Medicine and supply logistics	4	Digital stock tracker, reorder levels, barangay buffer budget
Records management	4	Active-chart index, archived inactive charts, EMR first
Reporting and compliance	4	Single data capture; digital submission
IT and equipment	4	Shared computers, backup internet, official workspace
Patient care	4	Patient queue; less double encoding
Other capabilities	3–4	Incremental improvements

2.6 Impacts to the Business Organization

- Clinical staff regain time from paperwork; an encoding support role may be needed.
- Aides and BHWs take on simple digital entry once equipped and trained.
- The Committee on Health gains a standing medicine buffer line in the annual budget.
- Paper remains as fallback during outages.

3. IT Capability Assessment

3.1 Baseline Maturity Level of Change Process

Level 2. System changes are decided by the City and its vendor; the EMR was introduced (around 2023) without consulting dentists and updated afterward. The center learns new systems through City seminars.

3.2 Target Maturity Level of Change Process

Level 3. The center records change requests, consults users before adopting tools, and sends formal requests to the City Health Department.

3.3 Baseline Maturity Level of Operational Processes

Level 2. Daily operations rely on manual logs, personal chats and one encoder; there is no backup connection.

3.4 Target Maturity Level of Operational Processes

Level 3–4. Official workspace, digital stock and report tracking, backup internet and documented fallback procedures.

3.5 Baseline Capability

Area	Baseline
Applications	City EMR (eMed), dental eMED, PhilHealth HCI/YAKAP, LMIS, ITIS, NNC e-OPT Plus, Google Drive, Messenger/Viber
Devices	4 City laptops (10+ years old), 2 barangay desktops, 1 barangay laptop, BNS barangay laptop, 4 printers (1 faulty), 1 phone
Connectivity	PLDT line paid by the barangay (~₱3,000 a month for the health center); LGU connection weak or unpaid
Support	One IT officer/encoder; barangay IT team; City vendor for the EMR

3.6 Capacity Assessment

The encoder handles EMR, PhilHealth and LMIS entries for all programs; the nurse does not use the EMR because of workload. Aides and BHWs (digital skills about 2/5, per Dr. Bonilla) have no devices. Capacity is the main constraint on further digital work.

3.7 Impacts to the IT Organization

- More devices and accounts to manage; the barangay IT team supports the center.
- Simple, documented procedures and basic training are needed.
- Formal coordination with the City for any EMR-related change.

4. Architecture Maturity Assessment

4.1 Architecture Governance Processes

No formal architecture governance exists at the center. Proposed: a small change board (Dr. Bonilla, Kagawad Rustia, Nurse Hardman) applying this project's principles; minor changes to the City Health Officer and major ones to the City Mayor.

4.2 Organization, Roles, and Responsibilities

Role	No.	Employer / status	Name(s)
Medical Officer IV (Rural Health Physician)	1	City; permanent	Dr. Glynnis Lynn U. Bonilla
Rural Health Dentist	1	City; permanent (Tue–Thu here; Mon/Fri at Oranbo)	Dr. Joseph P. Magpali, DMD
Rural Health Nurse	1	City; permanent	Ms. Nita N. Hardman, RN
Rural Health Midwife	1	City; permanent	Ms. Mafe B. Abong, RM
Nutritionist-Dietitian (Nutrition Officer II)	1	City; permanent (Tue–Thu here)	Mr. Christian Jay G. Vasquez, RND
Barangay Nutrition Scholar	1	City; permanent since about 2021	Ms. Mary Anne M. Verches
IT Officer / Medical Encoder	1	City; permanent	Mr. Tito F. Reyes
Pasig Health Aide	4	City; job order (paid and hired by CHD)	Ms. Janneth D. Correa (OIC), Ms. Rowena A. Intalan, Ms. Rosemarie T. Labandero, Ms. Procelyn L. Tatoy
Barangay Health Worker	3	Barangay; contractual (paid and hired by the barangay)	Ms. Joan Marie E. Barcenas, Mr. Perfecto S. Pestaño Jr., Ms. Josefina G. Roa (equipment custodian)
Security Officer	1	Barangay	Mr. Jeson C. Medez
Utility	1	Barangay	Ms. Vilma D. Lendio

4.3 Architecture Skills Assessment

Clinical staff and the encoder rate about 5/5 in digital skills; aides and BHWs about 2/5 (OIC aide about 3–4/5). No architecture skills are in house; the UA&P team provides them for this project.

4.4 Landscape Definition

Defined for the first time by this project (Interview Findings and Phase B–D deliverables).

4.5 Standards Definition

Data Privacy Act (RA 10173); DOH 7-year record retention; PhilHealth YAKAP rules; DOH/NNC program guidelines (FHSIS, PPAN, First 1000 Days, OPT Plus validity of at least 80%).

4.6 Reference Model Definition

TOGAF Content Metamodel, tailored to 34 entities in the Preliminary Phase.

4.7 Assessment of Re-Use Potential

High: City EMR as system of record and its report exports; existing Google Drive habits; NNC e-OPT tool; barangay internet line; existing logbooks and forms as data designs; Community Wellness Pop-ups.

5. Business Transformation Readiness Assessment

5.1 Readiness Factors

Factor	Description
Vision and need	Staff consistently name the same problems
Sponsorship	Doctor, Kapitan and Kagawad engagement
Funding	Barangay budget cycle
Governance and approvals	City approval for systems; procurement speed
IT capacity and skills	Devices, encoder load, frontline skills
Willingness to change	Teamwork; openness to new tools
Ability to absorb change	Busy seasons and reporting cycles

5.2 Vision for Each Readiness Factor

Sustain strong sponsorship, tie improvements to the annual budget cycle (proposals due November 2026), start with changes inside the barangay's control, and prepare evidence-based requests for City-level changes.

5.3 Current Readiness Ratings

Factor	Current (1–5)	Notes
Vision and need	4	Clear, shared wish list
Sponsorship	4	Doctor will review; Kapitan willing to review and approve
Funding	3	Budgets exist; medicine buffer not yet a line item
Governance and approvals	2	City approval needed for systems; slow procurement
IT capacity and skills	2	One encoder; aides lack devices
Willingness to change	4	Strong teamwork; two aides said nothing needs changing
Ability to absorb change	3	OPT Plus Jan–Mar; month-end reporting

5.4 Target Readiness Ratings

Factor	Target (1–5)
Vision and need	5
Sponsorship	5
Funding	4
Governance and approvals	3
IT capacity and skills	3
Willingness to change	4
Ability to absorb change	3

5.5 Readiness Risks

Risk	Likelihood	Mitigation
City does not approve EMR–YAKAP changes	High	Do barangay-controlled packages first; submit evidence to CHD

Risk	Likelihood	Mitigation
2027 budget deadline (November 2026) passes before items are proposed	High	Brief the Kapitan and Kagawad with costed items
Medicine shortage recurs	Medium	Buffer budget and stock tracker
Staff overload during OPT Plus (Jan–Mar)	Medium	Schedule rollout outside OPT and month-end
Low digital skills of aides and BHWs	Medium	Short training; simple tools; UA&P workshops

A Abbreviations

Abbreviations, acronyms and reference codes used in this document and its figures, in alphabetical order.

Abbreviation	Meaning
ADM	Architecture Development Method, the step-by-step method of the TOGAF Standard
BHW	Barangay Health Worker
BNS	Barangay Nutrition Scholar
Brgy	Barangay
CHD	City Health Department (Pasig City)
DMD	Doctor of Dental Medicine
DOH	Department of Health
e-OPT	Electronic Operation Timbang Plus tool of the National Nutrition Council
eMed / eMED	Name of the City's electronic medical record system; “dental eMED” is its separate dental system
EMR	Electronic Medical Record
FHSIS	Field Health Services Information System (DOH)
HCI	Health Care Institution (PhilHealth portal for health facilities)
Hon.	Honorable (title of elected officials)
IPCR	Individual Performance Commitment and Review
IT	Information technology
ITIS	Integrated Tuberculosis Information System (DOH)
ITR	Individual Treatment Record (the patient's registration form)
LGU	Local Government Unit
LMIS	Logistics Management Information System
MR-SIA	Measles-Rubella Supplemental Immunization Activity
NCD	Non-communicable disease (e.g., hypertension, diabetes)
NGO	Non-government organization
NNC	National Nutrition Council
OIC	Officer-in-Charge
OPT / OPT Plus	Operation Timbang Plus: yearly weighing and measuring of children aged 0–59 months
PhilHealth	Philippine Health Insurance Corporation
PLDT	PLDT Inc., an internet and telephone provider
PPAN	Philippine Plan of Action for Nutrition
RA	Republic Act (a Philippine law)
RM	Registered Midwife
RMC	Rizal Medical Center
RN	Registered Nurse
RND	Registered Nutritionist-Dietitian
SWMO	Solid Waste Management Office (City of Pasig)
TB	Tuberculosis
TOGAF	The Open Group Architecture Framework (the TOGAF® Standard)
UA&P	University of Asia and the Pacific
WP1 … WP9	Work package (a group of related changes delivered together)
YAKAP	PhilHealth Yaman ng Kalusugan Program (primary-care benefit)